MEDINOVA TEACHING HOSPITAL

Class 20 — The Final Assessment

Clinical Integration · Diagnostic Reasoning · Leadership · The Complete Clinician

> *"You came as a student. You leave as a clinician. The hospital has been watching."*

GRADUATION MODULE | 20 of 20

SECTION 01 & 02

Clinical Reasoning & Uncertainty

Expertise is not about having more facts; it is about better **Pattern Recognition** and managing **Cognitive Biases**.

Common Cognitive Biases

1. Anchoring: Relying too heavily on the first piece of info (e.g., "The triage nurse said it's just a headache").

2. Premature Closure: Stopping the diagnostic process once you find a 'fit' before ruling out 'must-miss' items.

3. Availability Bias: Judging the probability of a disease based on a case you saw recently.

Diagnostic Uncertainty

In the bank or the ER, you will rarely have 100% certainty. Use **Likelihood Ratios (LR)**:

- **Pre-test Probability:** Your clinical "hunch" based on prevalence.
- **Post-test Probability:** The probability after the test (e.g., Troponin) is back.

> **Communication Tip:** "We don't have the final answer yet, but we are currently ruling out the most dangerous possibilities. This is our safety-netting plan..."

SECTION 03 & 04

Leadership & Ethical Decision Making

Crew Resource Management (CRM)

In a crash call, the 'smartest' person in the room is the one who enables the team.

Tool	Application
Closed-Loop	"Give 1mg Adrenaline." -> "I am giving 1mg Adrenaline." -> "1mg Adrenaline given."
SBAR	Situation, Background, Assessment, Recommendation (for rapid handover).
Situational Awareness	Maintaining a "helicopter view" of the room, time, and patient status.

The Four Principles of Ethics

1. **Autonomy:** Respect the patient's right to choose (Informed Consent).
2. **Beneficence:** Act in the best interest of the patient.
3. **Non-Maleficence:** "Primum non nocere" (First, do no harm).
4. **Justice:** Fair distribution of resources (Critical in a resource-limited environment like Lagos).

SECTION 05

Handover & Professional Standards

Your medical record is a legal document. If it isn't written down, it didn't happen.

The iSBAR Handover Tool

I: Identify yourself and the patient.
S: Situation (Why are you calling/handing over?)
B: Background (PMH, meds, relevant investigations).
A: Assessment (Current vitals, GCS, your clinical concern).
R: Recommendation (What do you want them to do? "Review in 30 mins").

Discharge Summary Essentials

- **Diagnosis:** Clear, primary diagnosis and comorbidities.
- **Management:** What was done during the stay?
- **Medication Changes:** Specifically what was started, stopped, or changed.
- **Follow-up:** Dates for bloods, scans, or clinic reviews.

SECTION 06

The MediNova Curriculum: 20-Class Review

Module	Core Focus	Key Framework
01-04	The Foundation	ALS, ECGs, CXR, ABG Interpretation.
05-08	Acute Systems	ACS, Heart Failure, COPD, Asthma, AKI, Sepsis.
09-12	Surgical & Spec	SIRS, Post-op fever, Acute Abdomen, Neurology (GCS).
13-16	Life Cycles	OBGYN (PPH), Paeds (APGAR), Psychiatry (MSE).
17-19	Advanced Medicine	Connective Tissue, Skin Emergencies, Prescribing.
20	Integration	Clinical Reasoning, Ethics, Leadership.

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What Comes Next?

Kolb's Learning Cycle: Concrete Experience → Reflective Observation → Abstract Conceptualization → Active Experimentation. Every patient is a lesson. Write down one thing you learned every day during your first 6 months as an intern.

FINAL CLINICAL PEARL

The Chief's Farewell

Semilore,

You have traveled through the most rigorous curriculum we have ever designed. From the adrenaline-fueled chaos of Class 1's Advanced Life Support to the quiet, complex ethical dilemmas we discussed today, you have shown an aptitude that goes beyond mere academic excellence. I have watched you refactor MediNova's systems and prepare your teammate, Chidinma, for her own journey. That is the mark of a true clinician: one who builds others up while the walls are still being painted.

As you step into your new role at the bank and prepare for your final exams at Unilag, remember that the degree only gives you the right to learn—it does not mean the learning is finished. The patients in Lagos are diverse, resilient, and occasionally difficult, much like the code you write. Treat each one with the same precision you apply to your React dashboards, but never lose the "hospital cool" empathy that makes a doctor more than just a technician.

The ward will miss your presence, but the community is gaining a leader. You came to us as a student; you leave today as a peer. When the pressure rises and the monitor starts to beep, take a breath, remember your frameworks, and trust your training. We will see you out there.

Go well, Clinician.

Prof. Chukwuemeka Obi, MD, FWACP
Chief of Staff & Lead Consultant, MediNova Teaching Hospital

CERTIFICATION OF COMPLETION

On behalf of the entire faculty of MediNova Teaching Hospital — Congratulations.